



PROCEDURE

Tele-mental Health Services

Scope (Staff):	All staff
Scope (Area):	CAMHS-wide

Child Safe Organisation Statement of Commitment

CAHS commits to being a child safe organisation by applying the National Principles for Child Safe Organisations. This is a commitment to a strong culture supported by robust policies and procedures to reduce the likelihood of harm to children and young people.

This document should be read in conjunction with this [disclaimer](#)

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Aim

To guide CAMHS staff in the use of tele-mental health across all CAMHS services where it has been deemed clinically appropriate and an option accepted by the parent or carer.

Risk

Failure to provide safe, high quality tele-mental health services for the treatment and care of children with mental health concerns.

Background

Telehealth is increasingly being used as a modality to deliver health care. However, the use of tele-mental health for specific clinical functions is governed by the [Mental Health Act 2014 \(WA\)](#) (the MHA).

The use of tele-mental health in CAMHS must be in accordance with the [CAHS Telehealth Service Policy](#); the MHA and the following applicable standards:

- [Chief Psychiatrist's Guideline for Audiovisual Communication](#)
- [National Safety and Quality Digital Mental Health Standards](#)
- [Professional Practice Standards and Guides for Telepsychiatry](#) (Royal Australian and New Zealand College of Psychiatrists)
- [Chief Psychiatrist's Standards for Clinical Care](#)

CAMHS clinical staff are also accountable to discipline specific standards of practice and codes of conduct when providing telehealth services. The Australian Health Practitioner Regulation Agency and National Boards provide additional [Telehealth guidance for practitioners](#).

Definitions

Additional participants: Additional participants in the tele-mental health consultation are to be considered based on the child's identified needs and the preferences of the child and/or parent/legal guardian. They may include, but are not limited to, carers, personal support persons, cultural support persons and interpreters.

Audio-visual communication: Under the MHA, this refers to the use of videoconferencing to provide "real-time, synchronous video and audio transmission between locations to bring people together". Audiovisual communications, for the purpose of the MHA, does not include audio only telecommunications. For the purpose of this policy the term "Telehealth" or "Tele-mental health" is considered synonymous with audiovisual communication.

Carer: A person who provides care or assistance to a person who meets the criteria in s 5(1) of the [Carers Recognition Act 2004 \(WA\)](#). For a child of Aboriginal or Torres Strait Islander descent, this may be a person from their kinship group who is recognised under customary laws and traditions as their carer (MHA s 281(2)(b))

Child: A person under 18 years of age (MHA s 4).

Parent: The person(s) who has parental responsibility for the child. Parental responsibility, in relation to a child, means all the duties, powers, responsibilities and authority which, by law, parents have in relation to children (MHA s 4, [Family Law Act 1975 \(Cth\)](#) (FLA) s 61B and [Family Court Act 1997 \(WA\)](#) (FCA) s 68). For the purposes of this guideline, this also includes any legal guardian of the child appointed under the [Guardianship and Administration Act 1990 \(WA\)](#) (GAA) (GAA s 3). Changes to relationships such as parental separation, divorce or remarriage do not alter parental responsibility, unless there is a specific court order changing parental responsibility (FLA s 61C(2), FCA s 69(2)).

Telehealth: The use of information and communication technology to provide healthcare over a distance. This includes the transmission of images, voice, data and videoconferencing between two or more sites.

Tele-mental health: For the purpose of this policy this term is considered synonymous “telehealth” and with audio-visual communication, the term used in the MHA.

Key points

- CAMHS provide specialist mental health consultation, liaison, crisis management and assessment via telehealth (tele-mental health). For more information refer to the [CAMHS Crisis Connect Entry Protocol](#) and [Community CAMHS Entry Protocol](#).
- In addition to the services offered by CAMHS, tele-mental health services may be offered as an option when it is safe, clinically appropriate and suitable for the child and their carer or parent.
- Tele-mental health consultations are not intended to replace home visits or community appointments in metropolitan regions.
- Children and their parent/carer invited to participate in a tele-mental health consultation must be:
 - thoroughly informed about the process; and
 - assured that tele-mental health services are optional and provided with other alternatives if applicable.
- When providing tele-mental health services, CAMHS clinical staff will comply with the same requirements expected when providing face-to-face services including;
 - patient identification in accordance with the [CAHS Patient/Client Identification Protocol](#);
 - obtaining consent for treatment in accordance with the [CAMHS Consent Guideline](#); and

- ensuring confidentiality of patient/client information in accordance with the [CAHS Confidentiality, Disclosure and Transmission of Health Information Policy](#).
- Where the child is of Aboriginal descent, the tele-mental health consultation must be conducted with an Aboriginal Mental Health Workers and/or significant members of the patient's community, including elders and traditional healers, unless it is not practicable to do so. Ongoing therapy appointments may not require additional support from Aboriginal Mental Health Workers and/or significant members of the patient's community, however this will be accommodated if requested.
- Additional participants in the tele-mental health consultation are to be considered based on the child's identified needs and preferences of the child and/or parent/carer. Staff must determine whether the child has any sensory or cognitive impairment, cultural requirements, safety concerns or other concern indicating the need for additional participants. They may include, but are not limited to, personal support persons, cultural support persons and interpreters.
 - Refer to the [CAHS System Language Services Policy](#) for more information regarding the engagement of interpreters.

Use of Telehealth in Mental Health Assessments and Examinations for Admission to an Authorised Hospital

- Under the MHA, an assessment or examination of a patient outside the metropolitan area may be conducted using Telehealth, if it is not practicable for the psychiatrist or practitioner and patient to be in each other's physical presence, and the patient being assessed is in the physical presence of a health professional [s. 48(3) and s. 79(3)].
- Mental Health examinations for which telehealth is not permitted in accord with the MHA:
 - When conducting an examination referred to in s. 77(c) (confirming an Inpatient Treatment Order) and s. 77(e) (before the review period for a Community Treatment Order ends).
 - Confirming an inpatient treatment order by a psychiatrist where the initial inpatient treatment order was made by a psychiatrist who was not in the physical presence of the patient during the examination as required by s. 68(3) or 124(3).
 - Completion of Form 6A from Community Treatment Order by a supervising psychiatrist for the purpose of making an inpatient treatment order as required by section 120(3).
- For more information refer to the table in **Appendix 1**.

Equipment Requirements

- CAMHS clinical staff must conduct tele-mental health consultations from a WA Health desktop computer, laptop, tablet, mobile device or from a video-conference-enabled meeting or consult room.
- Tele-mental health consultations must take place in a culturally secure location that facilitates privacy and confidentiality in accordance with all applicable legislation and the [CAHS Confidentiality, Disclosure and Transmission of Health Information Policy](#).
- The audio-visual communication platform must have a sufficiently consistent connection and be of sufficient quality for a consultation to take place.
- The technical integrity of the audio-visual communication platform should be of a level that ensures the security of the patient's privacy and data.
- Staff participating in the Tele-mental health consultation must be competent in the practical use of audio-visual equipment.
 - For training and technical support refer to the [CAHS Telehealth Service Policy](#) or contact the [CAHS Telehealth Service team](#).
- Clinical media resulting from a tele-mental health consultation is governed by the principles and standards outlined in the [CAHS Photography and Video / Audio Recording \(Clinical\) Policy](#) including obtaining written consent from the parent for the recording or capture of any clinical media.

Procedure

Prior to the Tele-mental Health Consultation

- Ensure the child and parent/carer has received the information about the process, privacy, confidentiality and any relevant technical aspects of the tele-mental health consultation including consent for the recording or capture of any clinical media in accordance with the [CAHS Photography and Video / Audio Recording \(Clinical\) Policy](#).
- Considering the potential clinical risk implications, ensure there is an alternative plan for the consultation if there is an equipment failure and that this is discussed with the child and parent/carer.
 - For example, a follow up phone call with the child and parent/carer where they may be advised to present to the nearest emergency department if risk indicates that this is the most clinically appropriate option.

During the Tele-mental Health Consultation

- The CAMHS clinician must confirm patient identity as per the [CAHS Patient/Client Identification Protocol](#)
- If consent to treatment is to be obtained via tele-mental health this must be done in accordance with the CAMHS Consent Guideline.

Termination of a Tele-mental Health Consultation

- The tele-mental health consultation can be terminated by the child, parent/carer, health professional or consultant psychiatrist for reasons of safety, audiovisual quality or any other factor likely to impede a successful consultation.
- In the event of an unplanned termination of the consultation for any reason, the consulting clinician will remain on standby to ensure the safety and wellbeing of the patient and attempt to contact the child and/or their parent/carer via another means (ie. via telephone).
- Any termination of a tele-mental health consultation is to be documented in PSOLIS/webPSOLIS/DMR and an appropriate plan put in place.

Documentation / Record keeping

- Tele-mental health consultations are to be appropriately recorded in the patient's healthcare record and PSOLIS/webPSOLIS/DMR systems.
- Ensure that it is documented at initial assessment that the child and parent/carer has been provided with written and verbal information in relation to the process, privacy, confidentiality and relevant technical aspects of the consultation.
- At a minimum other documentation must include:
 - That it was conducted by Telehealth, the reasons for the decision to use Telehealth.
 - The locations of the providing, receiving and additional sites and names of those in attendance at all sites.
 - Any factors impacting on the quality of the communication.
 - The outcome of the assessment or examination and resulting treatment plans.

Roles and Responsibilities

- The CAMHS clinician is responsible for following the procedures outlined in this document including documentation and record keeping or to ensure the procedure is followed if some tasks have been delegated to other CAMHS staff.
- The CAHS Telehealth Service will provide all technical support in the use of telehealth-mental health, including training.

Monitoring and Reviewing

- Compliance will be monitored by CAMHS Directors, Heads of Department and/or Heads of Service and Clinical Nurse Consultants / Service Managers.
- Additional review of the services provided by telehealth will occur via YES/CES survey results.

Related CAHS internal policies, procedures and guidelines
CAHS Telehealth Service Policy
CAMHS Crisis Connect Entry Protocol
Community CAMHS Entry Protocol
CAHS Patient/Client Identification Protocol
CAMHS Consent Guideline
CAHS Confidentiality, Disclosure and Transmission of Health Information Policy
CAHS System Language Services Policy
CAHS Photography and Video / Audio Recording (Clinical) Policy
Acute Care and Response Team Model of Care
CAMHS Home and Community Visits Procedure

References and related external legislation, policies, and guidelines
Mental Health Act 2014 (WA)
Chief Psychiatrist's Guideline for Audiovisual Communication
National Safety and Quality Digital Mental Health Standards
Professional Practice Standards and Guides for Telepsychiatry (Royal Australian and New Zealand College of Psychiatrists)
Chief Psychiatrist's Standards for Clinical Care
AHPRA Telehealth guidance for practitioners
Carers Recognition Act 2004 (WA)
Family Law Act 1975 (Cth)
Family Court Act 1997 (WA)
Guardianship and Administration Act 1990 (WA)

This document can be made available in alternative formats on request.

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 Healthy kids, healthy communities CompassionExcellenceCollaborationAccountabilityEquityRespect Neonatology Community Health Mental Health Perth Children's Hospital			

Appendix 1: Legislated Use of Telehealth for Mental Health Act Assessments and Examinations

Group	Form No.	Form Title	Conditions for Use of Telehealth for Mental Health Act Examinations in CAHS (In addition to non-metropolitan area and not practicable to be in one another's physical presence)
Referrals	1A	Referral for Examination by a Psychiatrist	AV may be used: - By a Medical Practitioner or AMHP to assess a person for referral for examination by a Psychiatrist (Complete a Form 1A) where the person is not in an authorised hospital - By a Psychiatrist to conduct an examination of a person on a Form 1A (Referral for Examination by a Psychiatrist) where the person is not in an authorised hospital
Voluntary Inpatient	2	Order to Detain a Voluntary Inpatient in an Authorised Hospital for Assessment	AV may be used to conduct an assessment of a person on a Form 2 if the person being assessed is not in an authorised hospital
Detention	3A	Detention Order	AV may be used to complete a Form 3A detaining a person in order to take them to an authorised hospital
	3B	Continuation of Detention	AV may be used to complete a Form 3B extending a period of detention in order to take them to an authorised hospital
	3C	Continuation of Detention Following Examination	AV cannot be used to examine a person on a 1A in an authorised unit with a view to continuation of detention under a 3C AV cannot be used to examine a person on a 3C in an authorised unit with a view to completing a 6A
	3D	Order Authorising Reception and Detention in an Authorised Hospital for Further Examination	AV may be used to complete a Form 3D authorising reception and detention at an authorised hospital for further examination
	3E	Order That a Person Cannot Continue to be Detained	AV may be used if the person being assessed is not in an authorised hospital
CTO	5A	Community Treatment Order	- A psychiatrist may conduct an examination via AV for the purpose of completing a CTO – coach checking re APU - The supervising psychiatrist, medical practitioner or AMHP may conduct an examination via AV for the purpose of confirming the CTO - The supervising psychiatrist may conduct an examination via AV for the purpose of revoking the CTO
	5B	Continuation of a Community Treatment Order	The supervising psychiatrist may conduct an examination via AV for the purpose of deciding if the CTO should be continued
	5D	Request Made by a Supervising Psychiatrist for a Practitioner to Conduct the Monthly Examination	A practitioner may use AV to conduct a monthly examination of a patient on a CTO as requested by the supervising psychiatrist by completing a form 5D where the person is not in an authorised hospital
ITO	6A	Inpatient Treatment Order in an Authorised Hospital	AV may be used to conduct an examination of a person on a CTO in the community and complete a form 6A, however, where the patient has not had a face-to-face examination since the 6A was completed via audiovisual, a Confirmation of Inpatient Treatment Order (Form 6D) must be completed face to face within 24 hours of admission to an authorised hospital.
	6B	Inpatient Treatment Order in a General Hospital	AV may be used where the person is not in an authorised hospital
	6D	Confirmation of Inpatient Treatment Order	AV cannot be used to complete a Confirmation of Inpatient Treatment Order (Form 6D) where the examination to complete the 6A was also completed via AV
Other	-	Further Opinion	AV may be used to conduct an examination of a person for the purpose of providing a Further Opinion regardless of whether they are in an authorised unit.

